

Direct Primary Care

Continuity with your physician.
Seven-day access to the team.

One owner, two associates, assigned routine panels
and protected short-notice appointments.

An operating hypothesis for physician and investor review.

Relationships and access are complementary promises

Assigned continuity

One included routine visit each month, with the member's assigned physician on any of that doctor's clinic days. The concept encourages a regular relationship and community habit.

Team access

Additional, short-notice and semi-urgent needs can be seen by another scheduled team doctor. Seven-day coverage does not mean every doctor is available every day.

No practice endorsement, agreed physician participation, measured outcomes or paid demand is implied.

Consecutive days off.
Intentional Tuesday-Thursday overlap.

Role	Sun	Mon	Tue	Wed	Thu	Fri	Sat
Physician owner	Off	Off	On	On	On	Off	Off
Associate A	On	On	On	On	On	Off	Off
Associate B	Off	Off	On	On	On	On	On
Onsite total	1	1	3	3	3	1	1

No on-call obligation on days off. Collaboration and mentoring require protected time when scheduled; the overlap is not free teaching capacity.

Protect access before routine booking

Illustrative clinic day	Owner	Each associate
Onsite / available	10 a.m.-7 p.m.	9 a.m.-6 p.m.
Routine window	10 a.m.-5 p.m.	9 a.m.-6 p.m.
Break / admin / reserve	1 / 1 / 1 hours	1 / 1 / 1 hours
Routine inventory / total ceiling	6 / 7 encounters	9 / 12 encounters
Working weeks / unscheduled leave	46 / 6	46 / 6

No late-evening service is assumed. Owner reserve 5-6 and admin 6-7 sit outside routine bookings. Additional mentoring reduces routine inventory.

Illustrative assumptions for expert review. No practice affiliation, operating data or commitments implied.

R09 corrects the care policy, not just the calendar

Revision	Members	Revenue	Surplus
R07 published	360	\$1,263,600	\$286,080
R08 draft	247	\$866,970	-\$39,030
R09 review	345	\$1,210,950	\$243,960

R08 fixed weekday pooled demand did not match the intended policy. R09 uses assigned panels and 100% attendance, with protected reserve separate from routine scheduling. This is not an isolated roster comparison.

Budget the intended monthly habit

Assumption	Selected base
Included routine entitlement	One visit per member per month
Completed routine attendance	100%; 70% shown separately
Core / Extended routine block	25 / 45 minutes
Long-visit share / average block	75% / 40 minutes
Replacement onboarding / monthly attrition	45 minutes / 1%

Attendance, visit mix, no-shows and patient preference are assumptions, not measured utilization. Onboarding is additional work, with no invented fee.

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345 supported members
at 100% monthly attendance

Physician	Raw slots/mo	Panel limit	Assigned
Physician owner	69	57	57
Associate A	172.5	144	144
Associate B	172.5	144	144

360 requested; 345 modeled. Starting allocation: 16.7% owner and 41.7% each associate. Whole-person rounding yields 57 / 144 / 144. These are planning allocations, not observed preferences.

Held time is not a completed encounter

Day	Doctors	Held	Expected	Unused hrs	Excess
Sunday	1	3.83	1.44	2.40	0.00
Monday	1	3.83	2.88	0.96	0.00
Tuesday	3	11.50	2.88	8.62	0.00
Wednesday	3	11.50	2.88	8.62	0.00
Thursday	3	11.50	2.88	8.62	0.00
Friday	1	3.83	2.88	0.96	0.00
Saturday	1	3.83	1.44	2.40	0.00

Monthly totals by weekday, averaged over 46 working weeks. Base: five requests per 100 members/month, 60-minute blocks, double weekday weight.

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Annual averages cannot prove same-day or next-day service

What the model tests

Assigned routine panels, weekday workload, protected reserve, encounter ceilings, onboarding, uneven preferences, working weeks, staffing stages and off-day escalation conflicts.

What operations must prove

Arrival timing, acuity, no-shows, cancellations, appointment choice, dated leave, documentation burden and actual response performance. Sunday, Monday, Friday and Saturday have one doctor.

15% routine scheduling buffer is applied once. Reserve is held separately. Relief dollars do not create a substitute schedule.

Keep pricing and payment terms explicit

Tier	Monthly	Annual upfront
Core / 25-minute routine	\$250	\$2,400
Extended / 45-minute routine	\$350	\$3,360

20% annual discount. At 25% Core / 75% Extended and 50% annual take-up, earned dues average \$292.50/member/month.

Short-notice inclusion versus existing extra-visit charges is unresolved. Base short-notice revenue: \$0. Separately entered paid visits consume reserve and must be distinct encounters.

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Earned revenue funds the practice

Annual base economics	USD
345 members x \$292.50 x 12	\$1,210,950
Base costs including coverage allowances	\$906,000
Bonus employer-cost pool	\$60,990
Total expenses	\$966,990
Operating surplus / margin	\$243,960 / 20.1%

Break-even: 259 members. 15% target margin: 318. Before tax, debt and startup recovery; paid enrollment remains unproven.

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Existing salaries remain the starting case

Compensation / expense assumption	USD per year
Owner base / each associate base	\$180,000 / \$150,000
Gross bonus per associate	\$25,413
Total cash per associate	\$175,413
Employer burden on all gross bonuses	\$10,165
Provisional additional coverage allowances	\$60,000

20% of positive membership-funded surplus, inclusive of employer bonus costs; \$0 hurdle, full quality award. Not agreed terms. No product or prescribing commission.

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Show the downside as clearly as the target case

Case	Members	Revenue	Surplus
250; all annual pay	250	\$780,000	-\$126,000
Base; 100% attendance	345	\$1,210,950	\$243,960
360; 70% attendance	360	\$1,263,600	\$286,080
40% owner-panel demand	143	\$501,930	-\$404,070

Upside depends on lower attendance and paid demand. Owner-heavy allocation holds unused associate capacity instead of silently switching routine physicians.

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Upfront cash is not operating profit

Annual-pay share	Earned revenue	Bonus / associate	Surplus
0%	\$1,345,500	\$36,625	\$351,600
50%	\$1,210,950	\$25,413	\$243,960
100%	\$1,076,400	\$14,200	\$136,320

Cash is collected at joining and renewal; revenue is earned monthly. Cancellations refund unused prepaid months in this model. Deferred dues remain a service/refund obligation; final contract terms are unresolved.

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Grow each panel without assuming reassignment

Launch assumption	Reference
Owner / A / B start	Months 2 / 5 / 11
Opening / monthly later applicants	30 / 25
Opening admitted / waiting	5 / 25
Panel weights	16.7% / 41.7% / 41.7%
End of month 36	345 members

Applicants wait for their assigned doctor. Joining doctors do not take an existing panel. Limited staffed-day launch access requires patient acceptance.

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Operating recovery and funding are different questions

36-month result	Reference
Cumulative operating result	\$99,533
Sustained monthly break-even	Month 12
Cumulative operating-loss recovery	Month 32
Startup / opening cash / reserve	Unknown
Complete funding requirement	Unknown

Funding and upfront annual receipts are not earnings. Cash timing, startup spending, debt, financing and reserves remain separately visible in the worksheet.

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A junior is a separate scenario

Roster	Supported	Modeled	Surplus
Owner + two associates	345	345	\$243,960
Add Fri-Mon junior	460	360	\$142,080

Junior: \$150,000 plus employer burden. Additional rooms and support are uncoded. Its proposed panel is a scenario, not consent to reassign existing members. Incremental demand must justify the hire.

Validate one practice before expanding

Physician evidence

Recruitment acceptance, actual daily workload, protected mentoring, leave coverage, quality, continuity and compensation terms. Ownership and leadership progression require funded clinical time reductions.

Commercial evidence

Paid willingness to buy the premium offer, assigned-panel preferences, attendance, retention, collections and costed support. Improved outcomes and retention are hypotheses, not claims.

Shared support fee remains \$5,000/location/month. Platform delivery costs and central overhead remain unknown. No ancillary revenue or investor return is invented.

Scope, payment and legal terms still require expert decisions

Membership terms

Define which additional encounters are included, whether existing extra-visit charges apply, cancellation/refund details, actual response terms and launch access. No unlimited included visits are promised.

Category context: AAFP

Clinical and ownership scope

No expanded services or physician commitments. Existing optional-care discussion does not add base revenue. Ownership, clinical boundaries, fee arrangements and tax treatment need qualified review.

DPC definition: DPC Frontier

A reproducible reference, with earlier work preserved

Page, calculator and both PDF paths use the same R09 reference snapshot. Existing saved worksheets keep their names, inputs, notes, feedback and historical engines. Start R09 explicitly to retain the current worksheet and open the revised care model.

R07 is published. R08 was a roster-only review; R09 supersedes it within draft PR #56. Neither review is counted as a published release. No deployment is authorized by this document.

Review PR #56

Current published page (R07)